MATERNAL & NEWBORN HEALTH NURSING · COMPLETE QUESTION BANK

Maternity Nursing
All Lectures Question Bank

تمريض النسا — مجمّع أسئلة المادة كلها

9	243	72	127
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المصادر:

ميدترم 2022–2023 · ميدترم 2021–2022 · كويز 1 – 2023 · كويز 2 – 2022 · كويز 1 – 2022 · فاينال 2019–2020 (بنك الأسئلة) · كويز 1 – 2020 · كويز 1 – 2021 · كويز 2 – 2021 (نسخة أ) · كويز 2 – 2021 (نسخة ب) · كويز طب نسا · كويز تجريبي (فورم) · أسئلة سابقة

ترقيم الأسئلة: رقم المحاضرة ثم رقم السؤال (مثلاً 4.12). أسئلة NCLEX عليها حرف N. الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

LECTURE 1

Anatomy of the Female Reproductive System

تشريح الجهاز التناسلي الأنثوي

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1.1 This is the portion of the uterus that opens into the vagina: — ميدترم 2023–2022

a. Urethra
b. Cervix
c. Uterine tubes
d. Inguinal canal

1.2 _____ is the ligament that maintains the anteverted position of the uterus. — ميدترم 2023–2022

a. Ovarian ligament
b. Round ligament
c. Broad ligament
d. Cervical ligaments

1.3 The part of the uterus that lies above the body of the uterus is called: — ميدترم 2023–2022

a. Cervix
b. Fundus
c. Corpus
d. Isthmus

1.4 The vagina lies between the bladder anteriorly and the rectum posteriorly. (True / False) — ميدترم 2022–2021

a. True
b. False

1.5 Bartholin's glands open into the: — ميدترم 2022–2021

a. Fallopian tubes and release a secretion that makes sperms motile
b. Uterus and release a lubricating fluid during birth
c. Vestibule and release a lubricating fluid in the vagina
d. Urinary bladder and assist in release of urine

1.6 Which type of pelvis is rounded in shape, ideal for vaginal delivery and considered the classic female pelvis? — كويز 1 – 2023

a. Gynecoid
b. Android
c. Anthropoid
d. Platypelloid

1.7 Which of the following organs are included in the internal reproductive system? — كويز 1 – 2023

a. Ovaries, fallopian tubes and clitoris
b. Ovaries, vestibule and Bartholin's glands
c. Fallopian tubes, ovaries, vagina and labia majora
d. Fallopian tubes, uterus, vagina and ovaries

1.8 Which uterine wall layer is shed during menstruation? — كويز 1 – 2023

a. Endometrium
b. Myometrium
c. Perimetrium
d. Endothelium

1.9 The protective substance which protects the umbilical cord is called Bartholin's gland. (True / False) — كويز 1 – 2023

a. True **b.** False

1.10 Skene's ducts open on either side of the vaginal orifice. (True / False) — كويز 1 – 2023

a. True **b.** False

1.11 The organ that acts as a safety precaution against foreign bodies entering the uterus: — كويز 1 – 2022

a. Vestibule **b.** Vagina
c. Clitoris **d.** Cervix

1.12 _____ is the layer of the uterus that is shed during menstruation. — كويز 1 – 2022

a. Endometrium **b.** Myometrium
c. Perimetrium **d.** Exometrium

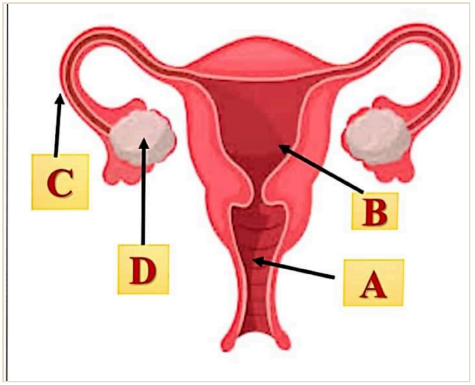
1.13 The muscle and tissue located between the vaginal opening and the anal canal is called: — كويز 1 – 2022

a. Perineum **b.** Vulva
c. Labia majora **d.** Urethra

1.14 The functions of the ovaries include secretion of: — كويز 1 – 2022

a. Estrogen and progesterone hormones **b.** LH hormone
c. Follicle-stimulating hormone (FSH) **d.** Prolactin hormone

1.15 What is the letter "A" referring to? — كويز 1 – 2021



a. Vagina **b.** Cervix
c. Uterus **d.** Fallopian tube
e. Ovary

1.16 What is the letter "B" referring to? (same figure above) — كويز 1 – 2021

a. Vagina **b.** Cervix
c. Uterus **d.** Fallopian tube
e. Ovary

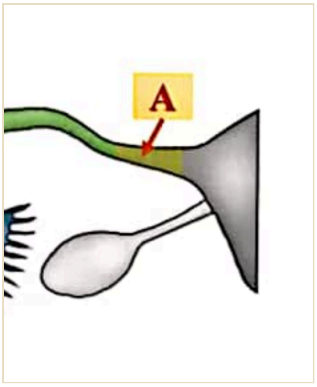
1.17 What is the letter "C" referring to? (same figure above) — كويز 1 – 2021

a. Vagina
b. Cervix
c. Uterus
d. Fallopian tube
e. Ovary

1.18 What is the letter "D" referring to? (same figure above) — كويز 1 – 2021

a. Vagina
b. Cervix
c. Uterus
d. Fallopian tube
e. Ovary

1.19 What is the letter "A" referring to? — كويز 1 – 2021



a. Interstitial part
b. Isthmus
c. Ampulla
d. Infundibulum
e. Fimbriae

1.20 What is the letter "B" referring to? (same figure above) — كويز 1 – 2021

a. Interstitial part
b. Isthmus
c. Ampulla
d. Infundibulum
e. Fimbriae

1.21 What is the letter "C" referring to? (same figure above) — كويز 1 – 2021

a. Interstitial part
b. Isthmus
c. Ampulla
d. Infundibulum
e. Fimbriae

1.22 What is the letter "D" referring to? (same figure above) — كويز 1 – 2021

a. Interstitial part
b. Isthmus
c. Ampulla
d. Infundibulum
e. Fimbriae

1.23 Fallopian tubes are directed from the ovary to the uterus in the following order: — كويز 1 – 2020

a. Interstitial, ampulla, isthmus, infundibulum
b. Infundibulum, ampulla, isthmus, interstitial
c. Interstitial, isthmus, ampulla, infundibulum
d. Isthmus, ampulla, interstitial, infundibulum

1.24 Which option includes all of the bones that make up the pelvis? كويز 1 – 2020

a. 2 innominate, sacrum and coccyx
b. 2 ilium, ischium and pubis
c. Innominate, 2 sacrum and coccyx
d. Trochanter, sacrum, coccyx

1.25 Which structures are finger-like projections at the superior pole of the ovary? فاينال 2019–2020 (بنك الأسئلة) تكرر في 2 مصادر

a. Ampulla
b. Corpus luteum
c. Corpus albicans
d. Fimbriae of uterine tube

1.26 Where in the female reproductive system will you find thousands of follicles? فاينال 2019–2020 (بنك الأسئلة) تكرر في 2 مصادر

a. Vagina
b. Fallopian tube
c. Ovaries
d. Uterus

1.27 The entrance to the uterus is also known as the: تكرر في 2 مصادر فاينال 2019–2020 (بنك الأسئلة)

a. Ovary
b. Cervix
c. Fallopian tube
d. Vagina

1.28 Which structures are finger-like projections at the superior pole of the ovary? أسئلة سابقة تكرر في 2 مصادر

a. Ampulla
b. Corpus luteum
c. Corpus albicans
d. Fimbriae of uterine tube

1.29 Where in the female reproductive system will you find thousands of follicles? أسئلة سابقة تكرر في 2 مصادر

a. Vagina
b. Fallopian tube
c. Ovaries
d. Uterus

1.30 The entrance to the uterus is also known as the: تكرر في 2 مصادر أسئلة سابقة

a. Ovary
b. Cervix
c. Fallopian tube
d. Vagina

1.31 Which of the following is posterior to the vagina? أسئلة سابقة

a. Urethra and bladder
b. Perineal body and rectum
c. Pelvic floor muscles
d. The vulva

1.32 The muscle and tissue located between the vaginal opening and anal canal is called: كويز تجريبي (فورم)

a. Perineum
b. Vulva
c. Labia majora
d. Urethra

1.33 Which of the following are the internal organs of the female reproductive system? كويز تجريبي (فودم)

a. Uterus, bladder, vagina and appendix
b. Vagina, cervix, uterus, fallopian tubes and ovaries
c. Cervix, uterus, fallopian tubes and ovaries
d. Vagina, cervix, bladder, uterus, fallopian tubes and ovaries

1.34 Which hormone(s) is/are secreted from the ovaries? كويز تجريبي (فودم)

a. Estrogen and progesterone
b. Luteinizing hormone (LH)
c. Follicle-stimulating hormone (FSH)
d. Prolactin

1.35 The layer of the uterine wall that is shed during menstruation is: كويز تجريبي (فودم)

a. Myometrium
b. Perimetrium
c. Endothelium
d. Endometrium

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

1.N1 A client asks where the ovum is usually fertilized. The nurse's best response is: NCLEX

a. In the fimbriae
b. In the ampulla of the fallopian tube
c. In the isthmus
d. In the uterine cavity

1.N2 During a pelvic exam, the nurse explains which structure prevents ascending infection into the uterus. Which is correct? NCLEX

a. Vestibule
b. Cervix with its mucus plug
c. Labia minora
d. Hymen

1.N3 Which pelvic type is most favorable for a vaginal birth? NCLEX

a. Android
b. Anthropoid
c. Gynecoid
d. Platypelloid

1.N4 Select ALL structures that belong to the internal female reproductive organs. SELECT ALL

a. Vagina
b. Uterus
c. Fallopian tubes
d. Ovaries
e. Labia majora
f. Bartholin's glands

1.N5 A postpartum client has a perineal laceration. The nurse knows the perineum is located between: NCLEX

a. The clitoris and the urethra
b. The vaginal opening and the anus
c. The labia majora and labia minora
d. The urethra and the vagina

1.N6 Which uterine layer is responsible for contractions during labor? NCLEX

a. Endometrium
b. Myometrium
c. Perimetrium
d. Decidua

1.N7 **Which ligament keeps the uterus in its anteverted position?** NCLEX

a. Broad ligament
b. Ovarian ligament
c. Round ligament
d. Infundibulopelvic ligament

1.N8 **Bartholin's glands secrete mucus that:** NCLEX

a. Makes sperm motile in the tube
b. Lubricates the vaginal introitus
c. Protects the umbilical cord
d. Is released during urination

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Ampulla	Widest, longest part of the tube — usual site of fertilization	**Isthmus (tube)**	Narrow part of the tube next to the uterus
Infundibulum / Fimbriae	Funnel end of the tube with finger-like fimbriae over the ovary	**Order of the tube (ovary ›› uterus)**	Infundibulum → Ampulla → Isthmus → Interstitial
Endometrium	Inner uterine layer shed during menstruation	**Myometrium**	Muscle layer — contractions of labor
Fundus	Part of the uterus above the openings of the tubes / above the body	**Cervix**	Lower part of the uterus opening into the vagina; barrier against infection
Perineum	Tissue between vaginal opening and anus	**Bartholin's glands**	Open into vestibule; lubricate the vagina
Skene's glands	Paraurethral glands that open on either side of the urethral meatus	**Gynecoid pelvis**	Rounded; classic female pelvis; best for vaginal birth
Round ligament	Maintains anteversion of the uterus	**Pelvis bones**	2 innominate (ilium, ischium, pubis) + sacrum + coccyx

LECTURE 2

Menstrual Cycle

الدورة الشهرية والإخصاب

Part A · Past Exam Questions

أسئلة الدفعات السابقة

2.1 Menstruation takes place about two weeks following ovulation as a result of: — ميدترم 2023–2022

a. Secretion of estrogen
b. Decreasing FSH
c. Increasing LH
d. Decrease in both estrogen and progesterone

2.2 Ovulation occurs at _____ of the menstrual cycle. — ميدترم 2023–2022

a. The beginning
b. The middle
c. The end
d. The first ten days

2.3 The hormone secreted from the anterior pituitary in the first half of the cycle in response to maturation of the follicle inside the ovary: تكرر في 2 مصادر — ميدترم 2023–2022

a. GnRH
b. FSH
c. Estrogen
d. Prolactin

2.4 The logical order of fetal growth during pregnancy is: — ميدترم 2023–2022

a. Ovum, embryo, zygote, fetus
b. Zygote, ovum, embryo, fetus
c. Ovum, zygote, embryo, fetus
d. Zygote, ovum, fetus, embryo

2.5 The site of fertilization is the: — ميدترم 2023–2022

a. Uterus
b. Urethra
c. Uterine tube
d. Ovaries

2.6 The hormone that works with estrogen to prepare the endometrium for implantation of a fertilized egg is: — ميدترم 2022–2021

a. LH
b. FSH
c. HCG
d. Progesterone

2.7 Which of the following is cell division without growth? — ميدترم 2022–2021

a. Cleavage
b. Conjugation
c. Fertilization
d. Development

2.8 The extra-embryonic structure that provides nutrition to the fetus is: — ميدترم 2022–2021

a. Chorion
b. Placenta
c. Amnion
d. Umbilicus

2.9 Fertilization of sperm and ovum takes place in the: — میدترم 2022–2021

a. Ampulla of the oviduct
b. Isthmus of the oviduct
c. Fimbriae of the oviduct
d. Uterus

2.10 This usually occurs between days 13 and 15 of the menstrual cycle: — میدترم 2022–2021

a. Ovulation
b. Shedding of the uterine lining
c. The lining of the uterus builds up
d. The lining remains in place for a possible embryo

2.11 Which of the following is an effect of the increased level of estrogen in the proliferative phase of the menstrual cycle? — کویز 1 – 2023

a. Thickening of cervical mucus
b. Rupture of corpus luteum
c. Thickening of the endometrium
d. Decrease of FSH to zero

2.12 Which of the following hormones is secreted by the corpus luteum? — کویز 1 – 2023

a. LH
b. FSH
c. Human chorionic gonadotropin
d. Progesterone

2.13 Oogenesis is the production of mature sperm. (True / False) — کویز 1 – 2023

a. True
b. False

2.14 Implantation begins about the 6th–10th day after fertilization. (True / False) — کویز 1 – 2023

a. True
b. False

2.15 The blastocyst is composed of: — کویز 1 – 2022

a. Trophoblast and mesoderm
b. Trophoblast and inner cell mass
c. Trophoblast and ectoderm
d. Trophoblast and endoderm

2.16 The extra-embryonic structure that provides nutrition to the embryo is: — کویز 1 – 2022

a. Chorion
b. Placenta
c. Amnion
d. Umbilicus

2.17 The embryo begins implantation in the uterus: — کویز 1 – 2022

a. After 6 weeks
b. In the 3rd week
c. In the 2nd week
d. In the 4th week

2.18 Ovulation occurs at: — کویز 1 – 2022

a. The beginning of the menstrual cycle
b. The middle of the menstrual cycle
c. The end of the menstrual cycle
d. Any time during the cycle

2.19 The average menstrual cycle will repeat itself about every 28 days. (True / False) — کویز 1 – 2021

a. True
b. False

2.20 The hormone secreted from the anterior pituitary in the first half of the cycle in response to maturation of the follicle inside the ovary: تكرر في 2 مصادر

كويز 1 – 2021

a. GnRH
b. FSH
c. Estrogen
d. Prolactin

2.21 The first day of the menstrual period (menstruation) is counted as the 7th day of the menstrual cycle. (True / False)

كويز 1 – 2021

a. True
b. False

2.22 The hormone that increases sharply during the second half of the cycle and is responsible for maintaining the growth of the endometrial lining is:

كويز 1 – 2021

a. Estrogen
b. Progesterone
c. FSH
d. LH

2.23 Embedding of the fertilized ovum in the upper uterine segment occurs:

كويز 1 – 2020

a. 6–8 days after fertilization
b. 3–4 days after fertilization
c. 1 day after fertilization
d. 2 days after fertilization

2.24 The hormone that enhances ovarian production of progesterone during the second half of the menstrual cycle is:

كويز 1 – 2020

a. Growth hormone
b. FSH
c. LH
d. Adrenocorticotropic hormone

2.25 Union of a mature ovum and sperm to form a zygote means:

كويز 1 – 2020

a. Zygote
b. Morula
c. Fertilization
d. Implantation

2.26 During the first half of the cycle (days 1–14) the pituitary produces _____, which stimulates egg growth. تكرر في 2 مصادر

فاينال 2019–2020 (بنك الأسئلة)

a. Progesterone
b. LH
c. Estrogen
d. FSH

2.27 Just before ovulation, cervical mucus becomes _____ to promote passage of sperms into the uterus. تكرر في 2 مصادر

فاينال 2019–2020 (بنك الأسئلة)

a. Scant, thick and sticky
b. Thin, clear and elastic
c. Heavy, thin and sticky
d. Scant and thick

2.28 Mrs. N asks the nurse about the ovarian phases. The nurse tells her the phases occur in the following order: تكرر في 2 مصادر

فاينال 2019–2020 (بنك الأسئلة)

a. Luteal phase, follicular phase, menstrual phase
b. Proliferative phase, ovulation, secretory phase
c. Follicular phase, ovulation phase, luteal phase
d. Follicular phase, ovulation phase, secretory phase

2.29 During the first half of the cycle (days 1–14) the pituitary produces _____, which stimulates egg growth. تكرر في 2 مصادر

أسئلة سابقة

a. Progesterone
b. LH
c. Estrogen
d. FSH

2.30 Just before ovulation, cervical mucus becomes _____ to promote passage of sperms into the uterus. تكرر في 2 مصادر

أسئلة سابقة

a. Scant, thick and sticky
b. Thin, clear and elastic
c. Heavy, thin and sticky
d. Scant and thick

2.31 Mrs. N asks the nurse about the ovarian phases. The nurse tells her the phases occur in the following order: تكرر في 2 مصادر

أسئلة سابقة

a. Luteal phase, follicular phase, menstrual phase
b. Proliferative phase, ovulation, secretory phase
c. Follicular phase, ovulation phase, luteal phase
d. Follicular phase, ovulation phase, secretory phase

2.32 The first day of the menstrual period (menstruation) is counted as the 7th day of the female's menstrual cycle. (True / False)

أسئلة سابقة

a. True
b. False

2.33 During the menstrual cycle, what happens if no fertilized ovum is implanted in the uterus?

أسئلة سابقة

a. The corpus luteum stops producing hormones
b. Estrogen continues to grow the endometrium
c. The pituitary releases an increased level of LH
d. The corpus luteum starts secreting progesterone

2.34 The average menstrual cycle repeats itself about every 28 days. (True / False)

أسئلة سابقة

a. True
b. False

2.35 The hormone secreted from the anterior pituitary gland in the first half of the cycle in response to maturation of the follicle inside the ovary is:

أسئلة سابقة

a. GnRH
b. FSH
c. Estrogen
d. Prolactin

2.36 A student nurse accurately describes FSH and LH by stating that:

أسئلة سابقة

a. FSH and LH are released from the anterior pituitary gland
b. FSH and LH are secreted by the corpus luteum
c. FSH and LH are secreted by the adrenal glands
d. FSH and LH are released from the posterior pituitary

2.37 The fetal circulation (umbilical cord) consists of: أسئلة سابقة

a. Two umbilical veins and one artery
b. Two umbilical arteries and one vein
c. Arteries carrying oxygenated blood to the fetus
d. Veins carrying deoxygenated blood to the fetus

2.38 A woman asks the nurse in which part of the fallopian tube fertilization usually occurs. What is the correct response? كويز تجريبي (فورم)

a. Interstitial
b. Isthmus
c. Ampulla
d. Fimbria

2.39 A woman asks: "When does the fertilized ovum reach the uterine cavity?" The nurse's best response is: كويز تجريبي (فورم)

a. About 1 or 2 days after fertilization
b. About 3 or 4 days after fertilization
c. About 6 or 10 days after fertilization
d. About 8 or 9 days after fertilization

2.40 In a lecture on fertilization, which stage of development is the stage of 0 to 10 days' gestation? كويز تجريبي (فورم)

a. Fetal stage
b. Morula stage
c. Zygote stage
d. Embryonic stage

2.41 Which term describes the first menstrual cycle of the female? كويز تجريبي (فورم)

a. Menarche
b. Puberty
c. Adolescence
d. Pubescence

2.42 Each month a developing Graafian follicle produces estrogen which builds up the endometrium. This is called: كويز تجريبي (فورم)

a. The secretory phase
b. The premenstrual phase
c. The proliferative phase
d. The menstrual phase

2.43 In a regular 28-day cycle, ovulation occurs approximately on: كويز تجريبي (فورم)

a. Day 10
b. Day 14
c. Day 21
d. Day 28

2.44 If conception fails, menstruation occurs about two weeks after ovulation. This is because of: كويز تجريبي (فورم)

a. Enhanced secretion of estrogen
b. Inhibition of FSH
c. Stimulation of LH
d. Decrease in both estrogen and progesterone

2.45 Which gland produces FSH and LH? كويز تجريبي (فورم)

a. Hypothalamus
b. Anterior pituitary
c. Posterior pituitary
d. Adrenal glands

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

2.N1 A client with a 28-day cycle asks when she is most fertile. The nurse answers: NCLEX

a. Days 1–5
b. Around day 14
c. Around day 21
d. Day 28

2.N2 Which hormone surge directly triggers ovulation? NCLEX

a. FSH
b. Estrogen
c. LH
d. Progesterone

2.N3 After ovulation the cervical mucus becomes thick and sticky. This is mainly due to: NCLEX

a. Estrogen
b. Progesterone
c. FSH
d. Prolactin

2.N4 Select ALL that are effects of progesterone during the secretory phase. SELECT ALL

a. Endometrial glands become secretory
b. Basal body temperature rises
c. Cervical mucus becomes thin and elastic
d. Endometrium is maintained for implantation
e. FSH increases sharply

2.N5 If fertilization does not occur, menstruation follows because: NCLEX

a. LH rises
b. The corpus luteum degenerates and estrogen and progesterone fall
c. FSH falls to zero
d. hCG rises

2.N6 The fertilized ovum reaches the uterine cavity about: NCLEX

a. 1 day after fertilization
b. 3–4 days after fertilization
c. 10 days after fertilization
d. 3 weeks after fertilization

2.N7 Which statement about the corpus luteum is correct? NCLEX

a. It secretes FSH
b. It secretes mainly progesterone (and some estrogen)
c. It forms before ovulation
d. It is maintained by prolactin

2.N8 The blastocyst is composed of: NCLEX

a. Trophoblast and inner cell mass
b. Ectoderm and endoderm only
c. Morula and zygote
d. Chorion and amnion

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Menarche	First menstrual period
FSH	Anterior pituitary; stimulates follicle growth in the first half of the cycle
LH surge	Triggers ovulation; LH enhances progesterone production by the corpus luteum
Ovulation	Mid-cycle — about day 14 of a 28-day cycle (≈14 days before next menses)
Proliferative phase	Estrogen from the Graafian follicle builds the endometrium
Secretory phase	Progesterone matures and maintains the endometrium
Corpus luteum	Secretes progesterone after ovulation
Menstruation cause	Fall of both estrogen and progesterone when corpus luteum degenerates
Cervical mucus before ovulation	Thin, clear, elastic (spinnbarkeit)
Fertilization	Union of ovum and sperm forming a zygote
Cleavage	Cell division without growth
Morula reaches uterus	About 3–4 days after fertilization
Implantation	Begins ≈ day 6–7 (6–10) after fertilization
Blastocyst	Trophoblast + inner cell mass
Order of development	Ovum → zygote → embryo → fetus

LECTURE 3

Preconception Care

رعاية ما قبل الحمل

Part A · Past Exam Questions

أسئلة الدفعات السابقة

3.1 Lack of preconception services is considered a barrier. (True / False) — ميدترم 2022–2023

a. True **b.** False

3.2 What is the optimum time for preconception care? — كويز 1 – 2023

a. 1–2 weeks before the conceiving attempt
b. 1–2 months before the conceiving attempt
c. 3–6 weeks before the conceiving attempt
d. 3–6 months before the conceiving attempt

3.3 Which statement reflects one purpose of preconception care? — كويز 1 – 2022

a. Identify women who should not become pregnant
b. Encourage healthy lifestyles for couples desiring pregnancy
c. Ensure that women know about prenatal care
d. Ensure that pregnancy complications do not occur

3.4 Interconception is described as the period before the first pregnancy. (True / False) — كويز 1 – 2021

تكرر في 2 مصادر

a. True **b.** False

3.5 "Fit for pregnancy" is one of the preconception health concepts. (True / False) — كويز 1 – 2021

a. True **b.** False

3.6 Preconception care started one month before pregnancy is the best timing. (True / False) — كويز 1 – 2021

a. True **b.** False

3.7 Unwanted pregnancies are at greater risk for both maternal and fetal outcomes. (True / False) — كويز 1 – 2020

a. True **b.** False

3.8 Women with a previous history of fetal abnormality need to take folic acid before becoming pregnant. (True / False) — كويز 1 – 2020

a. True **b.** False

3.9 The minimum amount of folic acid supplementation a woman should use while planning pregnancy is 4 mg. (True / False) — فاينال 2019–2020 (بنك الأسئلة)

تكرر في 2 مصادر

a. True **b.** False

3.10 Preconception health only matters if you have health problems. (True / False) فاينال 2019–2020 (بنك الأسئلة)
تكرر في 2 مصادر

a. True **b.** False

3.11 Interconception is described as the period before the first pregnancy. (True / False) أسئلة سابقة
تكرر في 2 مصادر

a. True **b.** False

3.12 Preconception health only matters if you have health problems. (True / False) أسئلة سابقة
تكرر في 2 مصادر

a. True **b.** False

3.13 The minimum amount of folic acid supplementation a woman should use while planning pregnancy is 4 mg. (True / False) أسئلة سابقة
تكرر في 2 مصادر

a. True **b.** False

3.14 Which statement reflects the purpose of preconception care? كويز تجريبي (فورم)

a. Identify women who should not become pregnant
b. Encourage healthy lifestyles for couples desiring pregnancy
c. Ensure that pregnancy complications do not occur
d. Ensure that women know about prenatal care

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

3.N1 What is the optimal time to begin preconception care? NCLEX

a. 1 month before conception **b.** 3–6 months before a conception attempt
c. After the first missed period **d.** In the second trimester

3.N2 A woman planning pregnancy asks why she should start folic acid now. The nurse explains it mainly prevents: NCLEX

a. Gestational diabetes **b.** Neural tube defects
c. Preterm labor **d.** Anemia of pregnancy only

3.N3 Select ALL components of preconception care. SELECT ALL

a. Risk assessment **b.** Health promotion
c. Immunization (e.g., rubella) **d.** Management of chronic disease
e. Fetal non-stress testing

3.N4 **A woman with poorly controlled diabetes wants to conceive. The priority preconception advice is:** NCLEX

a. Conceive now and control glucose later
b. Achieve good glycemic control before conception
c. Stop all medications
d. Avoid all exercise

3.N5 **Interconception health stresses waiting at least how long between pregnancies?** NCLEX

a. 6 months
b. 12 months
c. 18 months
d. 36 months

3.N6 **Which vaccine must be given at least 1 month BEFORE pregnancy, not during it?** NCLEX

a. Tetanus
b. Influenza (inactivated)
c. Rubella (MMR)
d. Hepatitis B

3.N7 **Which statement shows the client understood preconception care?** NCLEX

a. It only matters if I have a health problem
b. It helps me become as healthy as possible before pregnancy
c. It starts after a positive pregnancy test
d. It replaces antenatal care

3.N8 **Lack of preconception services is best described as:** NCLEX

a. A risk factor
b. A barrier to preconception care
c. An intervention
d. A goal

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Preconception care	Interventions before pregnancy to optimize maternal and fetal outcomes	**Optimum timing**	3–6 months before conception attempt
Interconception	Period between pregnancies — wait ≥ 18 months	**Folic acid**	Taken ~3 months before pregnancy; prevents 50–70% of NTDs
Folic acid dose (course)	4 mg — especially with previous NTD/fetal anomaly	**Two practical ideas**	Planning pregnancy & Fit for pregnancy
Three life-course pathways	Before first pregnancy · Managing emerging risks · Next baby and beyond	**Purpose**	Encourage healthy lifestyles for couples desiring pregnancy
Barrier example	Lack of preconception services	**Unwanted pregnancy**	Higher maternal and fetal risks
Live vaccines	MMR / varicella before pregnancy — avoid during it	**Slogan**	Healthy women, healthy mothers, healthy babies

LECTURE 4

Physiological Changes During Pregnancy

التغيرات الفسيولوجية أثناء الحمل

Part A · Past Exam Questions

أسئلة الدفعات السابقة

4.1 Chloasma is softening of the lower uterine segment. (True / False) تكرر في 2 مصادر — ميدترم 2023–2022

a. True **b.** False

4.2 A change of the cardiovascular system that is NOT related to pregnancy is: — ميدترم 2023–2022

a. Increased systemic vascular resistance **b.** Decreased blood pressure
c. Increased blood volume **d.** Increased cardiac output

4.3 Linea nigra is a line of pigmentation between the umbilicus and the symphysis pubis. (True / False) — ميدترم 2023–2022

a. True **b.** False

4.4 The uterine muscle mass enlarges during the first trimester of pregnancy mainly because of: — ميدترم 2022–2021

a. Enlarged fetus **b.** Hypertrophy
c. Hyperplasia **d.** Increase of prostaglandin hormone

4.5 What hormone is responsible for the decrease in systemic vascular resistance during pregnancy? — ميدترم 2022–2021

a. Progesterone **b.** Estrogen
c. Vasopressin **d.** Human chorionic gonadotropin

4.6 Which urinary symptom does the pregnant woman most frequently experience during the first trimester? — ميدترم 2022–2021

a. Dysuria **b.** Frequency
c. Incontinence **d.** Burning

4.7 An increase in vaginal secretions during pregnancy is called leukorrhea and is caused by increased production of estrogen. (True / False) — ميدترم 2022–2021

a. True **b.** False

4.8 A 36-week gravid client complains of dyspnea when lying flat. The likely clinical reason is: — ميدترم 2022–2021

a. Maternal hypertension **b.** Fundal height
c. Polyhydramnios **d.** Congestive heart failure

4.9 A woman 6 weeks pregnant is having a vaginal examination. Which would the practitioner expect to find? ميدترم 2022–2021

a. Thin cervical muscle
b. An enlarged ovary
c. Thick cervical mucus
d. Pale pink vaginal wall

4.10 The mother may experience shortness of breath at the end of the third trimester. This is most probably the result of: كويز 1 – 2023

a. Normal occurrence because the fetus is using more oxygen
b. The fundus of the uterus is high, pushing the diaphragm upwards
c. An allergic reaction to the pregnancy hormones
d. Anemia of pregnancy

4.11 Chloasma is softening of the lower uterine segment. (True / False) تكرر في 2 مصادر كويز 1 – 2023

a. True
b. False

4.12 A physiological change of the cardiovascular system that does NOT occur during pregnancy is: تكرر في 2 مصادر فاينال 2020–2019 (بنك الأسئلة)

a. Increased systemic vascular resistance
b. Decreased blood pressure
c. Increased blood volume
d. Increased cardiac output

4.13 Chloasma is an irregular hyperpigmented area on the: تكرر في 2 مصادر فاينال 2020–2019 (بنك الأسئلة)

a. Breast, areola and nipples
b. Cheek, nose, forehead and neck
c. Abdomen, breast and thighs
d. Cheeks, forehead and nose

4.14 The increase of blood volume during pregnancy is made of: تكرر في 2 مصادر فاينال 2020–2019 (بنك الأسئلة)

a. Plasma only
b. Erythrocytes only
c. More plasma than erythrocytes
d. More erythrocytes than plasma

4.15 A physiological change of the cardiovascular system that does NOT occur during pregnancy is: تكرر في 2 مصادر أسئلة سابقة

a. Increased systemic vascular resistance
b. Decreased blood pressure
c. Increased blood volume
d. Increased cardiac output

4.16 Which rationale best explains why a pregnant woman should lie on her left side when resting or sleeping? أسئلة سابقة

a. To prevent compression of the vena cava
b. To facilitate digestion
c. To facilitate bladder emptying
d. To avoid fetal anomalies

4.17 The weight of the uterus on the inferior vena cava and iliac veins obstructs blood flow from the lower regions. This can lead to: أسئلة سابقة

a. Dependent edema
b. Physiologic anemia
c. Hemodilution
d. Varicose veins of the vulva only

4.18 Chloasma is an irregular hyperpigmented area on the: تكرر في 2 مصادر

أسئلة سابقة

a. Breast, areola and nipples
b. Cheek, nose, forehead and neck
c. Abdomen, breast and thighs
d. Cheeks, forehead and nose

4.19 The increase of blood volume during pregnancy is made of: تكرر في 2 مصادر

أسئلة سابقة

a. Plasma only
b. Erythrocytes only
c. More plasma than erythrocytes
d. More erythrocytes than plasma

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

4.N1 A woman at 30 weeks becomes dizzy when lying flat on her back. The nurse's first action is to: NCLEX

a. Elevate her legs
b. Turn her onto her left side
c. Give oxygen
d. Lower the head of bed

4.N2 A pregnant client's Hb is 10.8 g/dL at 28 weeks. The nurse explains this is most likely due to: NCLEX

a. Iron overdose
b. Hemodilution (plasma increases more than RBCs)
c. Bleeding
d. Decreased blood volume

4.N3 Select ALL normal cardiovascular changes of pregnancy. SELECT ALL

a. Increased cardiac output
b. Increased blood volume
c. Decreased systemic vascular resistance
d. Increased systemic vascular resistance
e. Slight rise in heart rate

4.N4 A client asks about the dark line on her abdomen. The nurse documents it as: NCLEX

a. Chloasma
b. Linea nigra
c. Striae gravidarum
d. Spider nevi

4.N5 Urinary frequency in the first trimester is mainly caused by: NCLEX

a. UTI
b. Pressure of the enlarging uterus on the bladder
c. Decreased renal blood flow
d. Glycosuria

4.N6 Which complaint at 36 weeks is expected and explained by the high fundus? NCLEX

a. Shortness of breath
b. Severe headache
c. Visual disturbances
d. Epigastric pain

4.N7 Increased vaginal discharge (leukorrhea) in pregnancy is caused by: NCLEX

a. Infection always
b. Increased estrogen
c. Decreased progesterone
d. Low pH only

4.N8 **Early uterine enlargement in the first trimester is mainly due to:** NCLEX

a. Hypertrophy only
b. Hyperplasia stimulated by estrogen
c. Fetal size
d. Amniotic fluid

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Blood volume	↑ 40–50%; more plasma than RBCs	**Physiologic anemia**	Hemodilution from plasma increase
Systemic vascular resistance	↓ due to progesterone	**Cardiac output**	↑ 30–50%
Supine hypotension	Uterus compresses vena cava → lie on left side	**Dependent edema**	Uterus obstructs venous return from lower limbs
Chloasma	Mask of pregnancy — cheeks, forehead, nose	**Linea nigra**	Dark line from umbilicus to symphysis pubis
Leukorrhea	↑ vaginal discharge due to ↑ estrogen	**Urinary frequency**	1st trimester (uterus on bladder) and late 3rd trimester
Dyspnea late pregnancy	High fundus pushes diaphragm up	**Uterine growth early**	Hyperplasia (estrogen); later hypertrophy
Cervix in early pregnancy	Thick mucus plug (operculum)		

LECTURE 5

Ethical & Legal Issues

القضايا الأخلاقية والقانونية

Part A · Past Exam Questions

أسئلة الدفعات السابقة

5.1 The principle of taking one's own decisions independently is called: — ميدترم 2022–2023

a. Beneficence
b. Justice
c. Non-maleficence
d. Autonomy

5.2 A situation requiring a choice between what appear to be equally desirable and/or undesirable alternatives is called: — ميدترم 2022–2023

a. Ethical dilemma
b. Ethical principles
c. Code of ethics
d. Ethical issue

5.3 The most important consideration regarding the information in informed consent is that it must be strictly in medical terms. (True / False) — ميدترم 2021–2022

a. True
b. False

5.4 Which ethical principle means "to be fair and equitable to all patients regardless of any service you want to give"? — كويز 1 – 2023

a. Beneficence
b. Veracity
c. Justice
d. Paternalism

5.5 When you help a pregnant mother with multiple fetal anomalies take the decision of induced abortion as it is a "benefits over risks" approach, this ethical approach is called: — كويز 1 – 2023

a. Utilitarian approach
b. Rights approach
c. Fairness (justice) approach
d. Common-good approach

5.6 A tool used by health-care institutions to document the occurrence of anything 'out of the ordinary' that has the potential to harm patients or employees: تكرر في 2 مصادر — كويز 1 – 2021

a. Competent delivery services
b. Informed consent
c. Incident reports
d. Reporting and documentation

5.7 Competent delivery services is usually defined through: — كويز 1 – 2021

a. Job description
b. Informed consent
c. Incident reports
d. Code of ethics

5.8 The ethical principle which means "to protect the weak person" is: — كويز 1 – 2021

a. Paternalism
b. Fidelity
c. Veracity
d. Justice

5.9 The principle of ethics that describes taking care of the patient by doing no harm is: كويز 1 – 2020

a. Beneficence
b. Justice
c. Non-maleficence
d. Respect for autonomy

5.10 Confidentiality means: كويز 1 – 2020

a. When the patient does not listen to the doctor
b. When financial resources are scarce and the patient is not compliant
c. When a patient authorizes disclosing it
d. For a patient who requires invasive treatment

5.11 Which of the following provides a framework to help nurses and others evaluate ethical issues? أسئلة سابقة

a. Ethical dilemma
b. Ethical principles
c. Legal issues
d. Code of ethics

5.12 The most important principle while caring for women is to ensure she takes her decisions independently. This principle is called: أسئلة سابقة

a. Autonomy
b. Justice
c. Beneficence
d. Non-maleficence

5.13 A tool used by health-care institutions to document the occurrence of anything 'out of the ordinary' that has the potential to harm patients or employees is: تكرر في 2 مصادر أسئلة سابقة

a. Competent delivery services
b. Incident reports
c. Reporting and documentation
d. Informed consent

5.14 When the nurse helps a pregnant mother with multiple fetal anomalies take the decision of induced abortion as a "benefits over risks" approach, this is called: كويز تجريبي (فورم)

a. Utilitarian approach
b. The rights approach
c. Maternal rights
d. Balance in human system

5.15 Which tool is used by health-care institutions to document the occurrence of anything 'out of the ordinary' that may harm patients or employees? كويز تجريبي (فورم)

a. Competent delivery services
b. Informed consent
c. Incident reports
d. Reporting and documentation

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

5.N1 A laboring woman refuses a recommended episiotomy after explanation. The nurse should: NCLEX

a. Proceed because it benefits her
b. Respect her decision (autonomy) and document
c. Ask her husband to sign
d. Sedate her

5.N2 **Before amniocentesis, the nurse's role regarding informed consent is to:** NCLEX

a. Explain the procedure risks in detail and obtain the signature as the provider
b. Witness the signature and confirm the client understood
c. Sign for the client
d. Skip consent because it is minor

5.N3 **Select ALL actions that maintain confidentiality.** SELECT ALL

a. Discussing the case only with the care team
b. Logging off the computer
c. Copying records for a relative without permission
d. Releasing information only with written consent
e. Discussing the client in the elevator

5.N4 **The principle 'obligation to tell the truth' is:** NCLEX

a. Fidelity
b. Veracity
c. Justice
d. Paternalism

5.N5 **A nurse gives oxytocin at the wrong rate and the client is harmed. This is an example of a lawsuit related to:** NCLEX

a. Failure to monitor
b. Medication problems
c. Confidentiality
d. Documentation only

5.N6 **Deciding on selective reduction in a multifetal pregnancy to give the greatest good to the greatest number reflects:** NCLEX

a. Rights approach
b. Utilitarian approach
c. Paternalism
d. Fidelity

5.N7 **Keeping a promise to stay with a laboring woman reflects which principle?** NCLEX

a. Fidelity
b. Beneficence
c. Justice
d. Veracity

5.N8 **A nurse who ethically opposes abortion asks to be excused from caring for a client having one. According to the ANA Code of Ethics:** NCLEX

a. She must care for the client
b. She may refuse but must ensure the client receives care
c. She can be fired
d. She must report the client

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Ethics	From Greek 'Ethos' — custom, habit, character
Autonomy	Right to make own decisions independently

Beneficence	Obligation to do good
Non-maleficence	Do no harm
Justice	Fair and equitable to all patients
Fidelity	Keeping promises / faithfulness
Paternalism	To protect the weak person
Veracity	Obligation to tell the truth
Ethical dilemma	Choice between equally desirable/undesirable alternatives
Utilitarian approach	Greatest balance of benefits over risks; greatest good for greatest number
Rights approach	Focus on individual's rights: choose, privacy, truth, freedom from harm
Informed consent	Required for invasive procedures — e.g., amniocentesis, internal fetal monitoring
Confidentiality	Information released only with patient's written permission
Incident report	Documents anything 'out of the ordinary' that may cause harm
Competent delivery services	Defined through the job description

LECTURE 6

Diagnosis of Pregnancy

تشخيص الحمل

Part A · Past Exam Questions

أسئلة الدفعات السابقة

6.1 Which of the following is a positive (sure) sign of pregnancy? — ميدترم 2023–2022

a. Fetal movement felt by the mother
b. Enlargement of the uterus
c. Positive pregnancy test
d. Positive ultrasound

6.2 Positive ballottement in the woman's prenatal record indicates: — ميدترم 2023–2022

a. Palpable contractions on the abdomen
b. Passive movement of the unengaged fetus
c. Fetal kicking felt by the client
d. Enlargement and softening of the uterus

6.3 Uterine soufflé is a sound made by the rushing of maternal blood to the placenta and its rate is the same as the woman's pulse. (True / False) تكرر في 2 مصادر — ميدترم 2023–2022

a. True
b. False

6.4 Which would the nurse do to examine for the presence of Chadwick's sign? — ميدترم 2023–2022

a. Auscultate for fetal heart sounds
b. Palpate the abdomen for fetal movement
c. Assess the cervix for color
d. Initiate a gentle upward tap on the cervix

6.5 In a multipara, quickening occurs after 20 weeks. (True / False) — ميدترم 2022–2021

a. True
b. False

6.6 Which of the following indicates a probable sign of pregnancy? — ميدترم 2022–2021

a. Uterine enlargement
b. Fetal heart rate detected by a non-electronic device
c. Outline of the fetus via radiography or ultrasound
d. Nausea and vomiting

6.7 An adaptation of pregnancy is increased blood supply to the pelvic region resulting in purplish discoloration of the cervix and vaginal mucosa, known as: — ميدترم 2022–2021

a. Ladin's sign
b. Hegar's sign
c. Goodell's sign
d. Chadwick's sign

6.8 Uterine soufflé is a sound made by the rushing of maternal blood to the placenta, and its rate is the same as the woman's pulse. (True / False) تكرر في 2 مصادر

ميدترم 2021–2022

a. True
b. False

6.9 A bluish discoloration of the vaginal mucosa means which of the following?

كويز 1 – 2023

a. Ladin's sign
b. Hegar's sign
c. Goodell's sign
d. Chadwick's sign

6.10 Painless, intermittent contractions of the uterus that begin early in pregnancy and continue throughout gestation are called: تكرر في 2 مصادر

فاينال 2019–2020 (بنك الأسئلة)

a. Goodell's sign
b. Ballottement
c. Hegar's sign
d. Braxton Hicks contractions

6.11 Which of the following is a probable sign of pregnancy? تكرر في 2 مصادر

فاينال 2019–2020 (بنك الأسئلة)

a. Amenorrhea
b. Goodell's sign
c. Fetal heart rate detected by an electronic device
d. Nausea and vomiting

6.12 At 8 weeks you note a bluish coloration of the mucous membrane of the cervix, vagina and vulva. You document this as: تكرر في 2 مصادر

فاينال 2019–2020 (بنك الأسئلة)

a. Hegar's sign
b. Goodell's sign
c. Chadwick's sign
d. A positive sign

6.13 The first fetal movement felt by the mother is known as: تكرر في 2 مصادر

فاينال 2019–2020 (بنك الأسئلة)

a. Lightening
b. Quickening
c. Ballottement
d. Engagement

6.14 A woman with regular menstruation comes to the clinic complaining of spotting blood two to three days before her expected menstruation. What is the appropriate answer?

أسئلة سابقة

a. It could be due to an ovarian tumor
b. It could be from implantation of the zygote
c. It is your normal menstruation
d. You may need to do an ultrasound

6.15 During assessment of a pregnant woman, the nurse notes a bluish discoloration of the cervix. The nurse documents this as:

أسئلة سابقة

a. Braxton Hicks contractions
b. Chadwick's sign
c. McDonald's sign
d. Goodell's sign

6.16 The instructor asks the student to describe quickening. Which statement indicates understanding? أسئلة سابقة

a. It is the irregular, painless contractions that occur throughout pregnancy
b. It is the fetal movement that is felt by the mother
c. It is the thinning of the lower uterine segment
d. It is the soft blowing sound heard when the uterus is auscultated

6.17 A pregnant woman reports irregular contractions; the nurse determines they are Braxton Hicks contractions. Which nursing action is most appropriate? أسئلة سابقة

a. Instruct the woman to contact the health care provider
b. Instruct her to maintain bed rest for the rest of pregnancy
c. Inform her that these contractions may occur throughout pregnancy
d. Call the maternity unit and inform them she is in prelabor

6.18 To test for the presence of ballottement, the nurse-midwife would: أسئلة سابقة

a. Auscultate for fetal heart sounds
b. Palpate the abdomen for fetal movement
c. Assess the cervix for thinning
d. Initiate a gentle upward tap on the cervix

6.19 Painless, intermittent contractions of the uterus that begin early in pregnancy and continue throughout gestation are called: تكرر في 2 مصادر أسئلة سابقة

a. Goodell's sign
b. Ballottement
c. Hegar's sign
d. Braxton Hicks contractions

6.20 Which of the following is a probable sign of pregnancy? تكرر في 2 مصادر أسئلة سابقة

a. Amenorrhea
b. Goodell's sign
c. Fetal heart rate detected by an electronic device
d. Nausea and vomiting

6.21 At 8 weeks you note a bluish coloration of the mucous membrane of the cervix, vagina and vulva. You document this as: تكرر في 2 مصادر أسئلة سابقة

a. Hegar's sign
b. Goodell's sign
c. Chadwick's sign
d. A positive sign

6.22 The first fetal movement felt by the mother is known as: تكرر في 2 مصادر أسئلة سابقة

a. Lightening
b. Quickening
c. Ballottement
d. Engagement

6.23 Goodell's sign indicates: أسئلة سابقة

a. Softening of the cervix
b. Soft blowing uterine sound synchronous with the maternal pulse
c. hCG in the urine
d. Fetal movement

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

6.N1 Which finding is a POSITIVE sign of pregnancy? NCLEX

a. Positive urine hCG
b. Hegar's sign
c. Fetal heart tones by Doppler
d. Quickening

6.N2 Softening of the lower uterine segment at 6–12 weeks is: NCLEX

a. Goodell's sign
b. Hegar's sign
c. Chadwick's sign
d. Ballottement

6.N3 Select ALL presumptive (subjective) signs of pregnancy. SELECT ALL

a. Amenorrhea
b. Nausea and vomiting
c. Breast tenderness
d. Quickening
e. Ballottement
f. Positive pregnancy test

6.N4 A false-positive pregnancy test may occur with: NCLEX

a. Hydatidiform (molar) pregnancy
b. Ectopic pregnancy only
c. Early ovulation
d. Dehydration

6.N5 Fetal heart tones can first be heard with a fetoscope at about: NCLEX

a. 6 weeks
b. 10–12 weeks
c. 20 weeks
d. 28 weeks

6.N6 A multigravida reports first feeling fetal movement. This most likely occurred at about: NCLEX

a. 8 weeks
b. 16 weeks
c. 28 weeks
d. 32 weeks

6.N7 The nurse taps upward on the cervix and feels the fetus rebound. This is: NCLEX

a. Quickening
b. Ballottement
c. Lightening
d. Engagement

6.N8 Funic soufflé corresponds to: NCLEX

a. The maternal pulse
b. The fetal heart rate
c. Uterine contractions
d. Placental separation

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Presumptive signs	Subjective — felt by the woman
Probable signs	Objective — found by examiner
Positive signs	Related to the fetus — confirm pregnancy
Chadwick's sign	Bluish/purplish cervix and vagina
Goodell's sign	Softening of the cervix
Hegar's sign	Softening of lower uterine segment, 6–12 weeks
Ballottement	Rebound of fetus — internal 16 wk, external 20 wk
Quickening	First fetal movement felt by mother — primi 20–25 wk, multi ~16 wk
Braxton Hicks	Painless irregular contractions throughout pregnancy
FHR by Doppler	10–12 weeks
FHR by fetoscope	~20 weeks
Uterine soufflé	Maternal blood to placenta; rate = mother's pulse
Funic soufflé	Blood in cord; rate = FHR
False-positive hCG	Molar pregnancy, trophoblastic tumors, pseudocyesis

LECTURE 7

Antenatal Care

رعاية الحامل (متابعة الحمل)

Part A · Past Exam Questions

أسئلة الدفعات السابقة

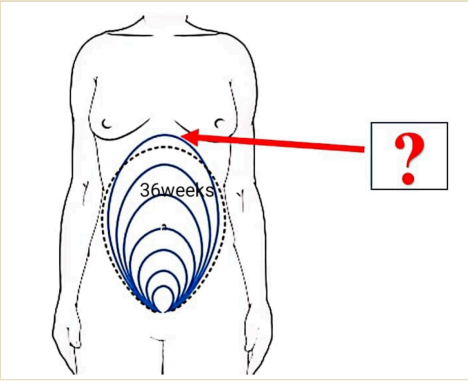
7.1 Which danger sign should be reported promptly during the antepartum period? — ميدترم 2023–2022

a. Constipation
b. Breast tenderness
c. Nasal stuffiness
d. Leaking amniotic fluid

7.2 Which fundal height indicates less than 12 weeks' gestation? — ميدترم 2023–2022

a. Uterus in the pelvis
b. Uterus at the xiphoid
c. Uterus in the abdomen
d. Uterus at the umbilicus

7.3 The midwife has just palpated the fundal height at the location noted on the picture (just below the xiphoid process). It is likely that the client is how many weeks pregnant? — ميدترم 2022–2021



a. 20 weeks
b. 28 weeks
c. 36 weeks
d. 40 weeks

7.4 A pregnant woman on her first antepartum visit has a 2-year-old son born at 40 weeks, a 5-year-old daughter born at 38 weeks, 7-year-old twin daughters born at 35 weeks, and a spontaneous abortion 3 years ago at 10 weeks. Her GTPAL is: — ميدترم 2022–2021

a. G4 T3 P2 A1 L4
b. G5 T2 P2 A1 L4
c. G5 T2 P1 A1 L4
d. G4 T3 P1 A1 L4

7.5 Gravida refers to which of the following descriptions? — ميدترم 2022–2021

a. A serious pregnancy
b. Number of times a female has been pregnant
c. Number of children a female has delivered
d. Number of term pregnancies a female has had

7.6 If the LMP is January 30, the expected date of delivery (EDD) is approximately: ميدترم 2022–2021

a. Oct. 7
b. Oct. 30
c. Nov. 7
d. Nov. 30

7.7 Shoes with low, broad heels plus good posture will prevent which prenatal discomfort? ميدترم 2022–2021

a. Backache
b. Vertigo
c. Leg cramps
d. Nausea

7.8 A pregnant client's LMP was September 10, 2019. Using Naegele's rule her EDD will be: تكرر في 2 مصادر فاينال 2020–2019 (بنك الأسئلة)

a. June 10, 2020
b. June 17, 2019
c. June 17, 2020
d. July 10, 2020

7.9 The nurse counsels a low-risk woman at 20 weeks' gestation to schedule her routine prenatal visits every: تكرر في 2 مصادر فاينال 2020–2019 (بنك الأسئلة)

a. 1 week
b. 2 weeks
c. 3 weeks
d. 4 weeks

7.10 To help reduce breast tenderness in the first trimester, the nurse tells the client to: أسئلة سابقة

a. Avoid wearing a bra
b. Wash nipples daily with soap and massage with lotion
c. Wear tight-fitting blouses for support
d. Wash the breasts with warm water and keep them dry

7.11 The first day of a client's last menstrual period was September 19, 2013. Using Naegele's rule, the estimated date of confinement is: أسئلة سابقة

a. July 26, 2013
b. June 12, 2014
c. June 26, 2014
d. July 12, 2014

7.12 A client pregnant with twins has a healthy 5-year-old delivered at 36 weeks 6 days and no history of abortion or fetal demise. Her GTPAL is: أسئلة سابقة

a. G3 T2 P0 A0 L1
b. G2 T0 P1 A0 L1
c. G1 T1 P1 A0 L1
d. G2 T0 P0 A0 L1

7.13 A pregnant client's LMP was September 10, 2019. Using Naegele's rule her EDD will be: أسئلة سابقة تكرر في 2 مصادر

a. June 10, 2020
b. June 17, 2019
c. June 17, 2020
d. July 10, 2020

7.14 The nurse counsels a low-risk woman at 20 weeks' gestation to schedule her routine prenatal visits every: أسئلة سابقة تكرر في 2 مصادر

a. 1 week
b. 2 weeks
c. 3 weeks
d. 4 weeks

7.15 Which finding in a primipara in the second trimester is abnormal and requires further testing? أسئلة سابقة

a. Consistent increase in fundal height
b. Fetal heart rate of 180 bpm
c. Braxton Hicks contractions
d. Quickening

7.16 A pregnant client's hemoglobin and hematocrit are low, indicating anemia. The nurse determines that the client is at risk for: أسئلة سابقة

a. A loud mouth
b. Low self-esteem
c. Hemorrhage
d. Postpartum infections

7.17 A nullipara is a woman who has delivered several times before. (True / False) أسئلة سابقة

a. True
b. False

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

7.N1 LMP is March 10. Using Naegele's rule, the EDD is: NCLEX

a. December 17
b. December 10
c. January 17
d. November 17

7.N2 A woman is pregnant for the 4th time; she had one term birth, one preterm birth (twins, both alive) and one miscarriage. Her GTPAL is: NCLEX

a. G4 T1 P1 A1 L3
b. G4 T2 P1 A1 L3
c. G3 T1 P1 A1 L2
d. G4 T1 P2 A1 L3

7.N3 Select ALL danger signs a pregnant woman should report immediately. SELECT ALL

a. Vaginal bleeding
b. Leaking of amniotic fluid
c. Severe headache with blurred vision
d. Decreased fetal movement
e. Mild ankle edema at night
f. Nasal stuffiness

7.N4 A low-risk woman at 32 weeks asks how often to come. The nurse answers: NCLEX

a. Every 4 weeks
b. Every 2 weeks
c. Every week
d. Only if problems occur

7.N5 Fundal height at the umbilicus corresponds to about: NCLEX

a. 12 weeks
b. 16 weeks
c. 20 weeks
d. 36 weeks

7.N6 To relieve backache in pregnancy, the nurse advises: NCLEX

a. High-heeled shoes
b. Low, broad-heeled shoes and good posture/pelvic tilt
c. Bed rest
d. Lifting with the back

7.N7 **A woman at 10 weeks complains of morning nausea. Best advice:** NCLEX

a. Drink large amounts of fluid with meals
b. Eat dry crackers before rising and small frequent meals
c. Skip breakfast
d. Lie flat after meals

7.N8 **At the first antenatal visit, which laboratory test is routine?** NCLEX

a. Blood group and Rh, CBC, urinalysis
b. Amniocentesis
c. Biophysical profile
d. Contraction stress test

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Naegele's rule	LMP − 3 months + 7 days (+1 year)	**Gravida**	Number of pregnancies (including current)
Para	Births after viability (≥ 20–24 wk); twins = 1	**GTPAL**	Gravida, Term, Preterm, Abortions, Living
Term	38–41 (37–42) weeks	**Visit schedule**	q4 wk until 28 · q2 wk 28–36 · weekly after 36
Fundal height 12 wk	At symphysis pubis	**Fundal height 20 wk**	At umbilicus
Fundal height 36 wk	At xiphoid	**< 12 weeks**	Uterus still in the pelvis
Backache relief	Low broad heels, good posture, pelvic tilt	**Danger signs**	Bleeding, leaking fluid, severe headache/blurred vision, ↓ fetal movement, fever
Breast tenderness	Supportive bra; wash with warm water only, keep dry		

LECTURE 8

Assessment of Fetal Well-being

تقييم سلامة الجنين

Part A · Past Exam Questions

أسئلة الدفعات السابقة

8.1 The nurse has performed a non-stress test and interprets it as reactive. The nurse documents this finding as: — ميدترم 2022–2023

a. Normal
b. Abnormal
c. Need for further evaluation
d. Findings were difficult to interpret

8.2 A pregnant client is scheduled for a biophysical profile. The test measures: — ميدترم 2022–2023

a. Mother's ability to tolerate pregnancy
b. Amniotic fluid volume and fetal activity
c. Mother's cardiac status and ability to tolerate labor
d. Only the amount of amniotic fluid

8.3 A biophysical profile score of 6 or below indicates fetal compromise. (True / False) — ميدترم 2022–2023

a. True
b. False

8.4 The best description of the contraction stress test is: — ميدترم 2022–2023

a. The uterus is stimulated to contract by small amounts of oxytocin or nipple stimulation
b. The woman ambulates until contractions begin
c. Uterine contractions are stimulated by Leopold's maneuvers
d. Uterine contractions before 28 weeks of gestation

8.5 On assessment of a mother at 38 weeks the fetal heart rate is 174 bpm. What is the best action? — ميدترم 2022–2023

a. Document the finding
b. Check the mother's heart rate
c. Notify the health-care provider
d. Tell the client the rate is normal

8.6 The nurse auscultates a fetal heart rate of 145 bpm in a mother in early labor. Which action is appropriate? — كويز 2 – 2022

a. Inform the mother that the rate is normal
b. Reassess in 5 minutes to verify the results
c. Immediately report the rate to the health-care practitioner
d. Place her on her left side and apply oxygen

8.7 The nurse auscultates a fetal heart rate of 152 bpm in a client in early labor. Which action is appropriate? كويز 2 – 2021 (نسخة ب)

a. Inform the mother that the rate is normal
b. Reassess in 5 minutes to verify the results
c. Immediately report the rate to the health-care practitioner
d. Place her on her left side and apply oxygen by face mask

8.8 Bradycardia refers to the average FHR during a 10-minute period that excludes periodic and episodic changes and periods of marked variability. (True / False) كويز 2 – 2021 (نسخة ب)

a. True **b.** False

8.9 Acceleration is a baseline FHR below 110 beats/min. (True / False) كويز 2 – 2021 (نسخة ب)

a. True **b.** False

8.10 Hypoxia is defined as insufficient availability of oxygen to meet the metabolic needs of body tissue. (True / False) كويز 2 – 2021 (نسخة ب)

a. True **b.** False

8.11 The onset, nadir and recovery of the deceleration occur after the beginning, peak and end of the uterine contraction in late deceleration. (True / False) كويز 2 – 2021 (نسخة أ)

a. True **b.** False

8.12 A pregnant woman asks the nurse to describe a contraction stress test. What should the nurse tell her? أسئلة سابقة

a. Uterine contractions are stimulated by small amounts of oxytocin
b. An external fetal monitor is attached and the woman ambulates until contractions begin
c. Uterine contractions are stimulated by Leopold's maneuvers
d. Uterine contractions are stimulated before 28 weeks of gestation

8.13 What is the priority nursing action when the fetal heart rate is 174 beats/minute? أسئلة سابقة

a. Document the finding
b. Notify the health care provider
c. Tell her the fetal heart rate is normal
d. Check the mother's heart rate

8.14 Which of the following is a characteristic of amniotic fluid? أسئلة سابقة

a. Can be used to measure fetal liver function
b. Prevents large particles such as bacteria from passing to the fetus
c. Provides exchange of nutrients and wastes between mother and fetus
d. Surrounds, cushions and protects the fetus

8.15 During the active stage of labor the nurse observes late decelerations on the monitor strip. What is the most appropriate nursing action? أسئلة سابقة

a. Administer oxygen via face mask
b. Place the mother in a supine position
c. Document the findings and continue to monitor
d. Increase the rate of the oxytocin infusion

8.16 At 38 weeks a client has late decelerations; the fetal pulse oximeter shows 75%–85%. The nurse should: أسئلة سابقة

a. Discontinue the catheter if the reading is not above 80%
b. Discontinue the catheter if the reading does not go below 30%
c. Advance the catheter until the reading is above 90%
d. Reposition the catheter, recheck, and if it is 55% keep monitoring

8.17 The nurse observes the client's amniotic fluid and decides it is normal because it is: أسئلة سابقة

a. Clear and dark amber
b. Milky, greenish-yellow with shreds of mucus
c. Clear, almost colorless, with little white specks
d. Cloudy, greenish-yellow with little white specks

8.18 After rupture of membranes the nurse notes variable decelerations in the fetal heart rate. The nurse should: أسئلة سابقة

a. Stop the oxytocin infusion
b. Change the client's position
c. Prepare for immediate delivery
d. Take the client's blood pressure

8.19 After Leopold's maneuvers the fetus is in ROP position. To best auscultate fetal heart tones, the Doppler is placed: أسئلة سابقة

a. Above the umbilicus at the midline
b. Above the umbilicus on the left side
c. Below the umbilicus on the right side
d. Below the umbilicus near the left groin

8.20 An elevation of 15 beats above a baseline of 135 bpm lasting 15 seconds should be documented as: أسئلة سابقة

a. An acceleration
b. An early elevation
c. A sonographic motion
d. A tachycardic heart rate

8.21 For a client delivering twins, the nurse places the external fetal monitor: أسئلة سابقة

a. Over the most anterior fetus
b. Over the most posterior fetus
c. So that each fetal heart rate is monitored separately
d. On one fetus for 15 minutes then the other for 15 minutes

أسئلة سابقة

8.22 **After attaching an external electronic fetal monitor on admission to the labor room, the initial nursing assessment is:**

a. Identifying the types of accelerations
b. Assessing the baseline fetal heart rate
c. Determining the frequency of contractions
d. Determining the intensity of contractions

أسئلة سابقة

8.23 **The nurse suspects umbilical cord compression if which of the following is noted on the external monitor tracing during a contraction?**

a. Early decelerations
b. Variable decelerations
c. Late decelerations
d. Short-term variability

أسئلة سابقة

8.24 **A client in active labor has contractions every 3 minutes lasting 45 seconds; the fetal heart rate between contractions is 100 bpm. The most appropriate action is:**

a. Encourage the coach to continue breathing exercises
b. Encourage the client to continue pushing
c. Continue monitoring the fetal heart rate
d. Notify the physician or nurse-midwife

أسئلة سابقة

8.25 **The nurse notes episodic accelerations on the electronic fetal monitor tracing. The most appropriate action is:**

a. Document the findings and tell the mother the monitor indicates fetal well-being
b. Take maternal vital signs and tell her bed rest is required
c. Notify the physician or nurse-midwife
d. Reposition the mother and check the tracing for changes

أسئلة سابقة

8.26 **Using a Doppler ultrasound device, the nurse most accurately determines that fetal heart sounds are heard by:**

a. Noting if the heart rate is greater than 140 bpm
b. Placing the diaphragm of the Doppler on the abdomen
c. Performing Leopold's maneuvers first
d. Palpating the maternal radial pulse while listening to the fetal heart rate

أسئلة سابقة

8.27 **A client at 36 weeks is scheduled for amniocentesis. Which statement about the preceding ultrasound indicates a need for further teaching?**

a. The ultrasound locates the placenta
b. The ultrasound shows the blood flow through the umbilical cord
c. The test determines the needle insertion site
d. The ultrasound locates the amniotic fluid

أسئلة سابقة

8.28 **What is a normal fetal heart rate at 38 weeks' gestation?**

a. 80 bpm
b. 100 bpm
c. 150 bpm
d. 180 bpm

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

8.N1 An NST shows two accelerations of 15 bpm lasting 15 seconds in 20 minutes. The nurse documents: NCLEX

- **a.** Nonreactive
- **b.** Reactive (reassuring)
- **c.** Positive
- **d.** Equivocal

8.N2 A BPP score of 4/10 indicates: NCLEX

- **a.** Normal
- **b.** Fetal compromise — notify provider
- **c.** Equivocal, repeat next week
- **d.** Maternal hypertension

8.N3 Late decelerations appear on the monitor. The FIRST nursing action is: NCLEX

- **a.** Increase oxytocin
- **b.** Turn the woman to her left side
- **c.** Document and continue to monitor
- **d.** Place her supine

8.N4 Select ALL components of the biophysical profile. SELECT ALL

- **a.** Fetal breathing movements
- **b.** Fetal body movements
- **c.** Fetal tone
- **d.** Amniotic fluid volume
- **e.** Reactive NST
- **f.** Maternal blood pressure

8.N5 Variable decelerations are caused by: NCLEX

- **a.** Head compression
- **b.** Uteroplacental insufficiency
- **c.** Umbilical cord compression
- **d.** Fetal sleep

8.N6 Normal baseline FHR is: NCLEX

- **a.** 90–110 bpm
- **b.** 110–160 bpm
- **c.** 160–180 bpm
- **d.** 60–100 bpm

8.N7 Daily fetal movement counting suggests compromise when the mother feels: NCLEX

- **a.** 10 movements in 2 hours
- **b.** Fewer than 10 movements in 12 hours
- **c.** Many movements in the evening
- **d.** Hiccups

8.N8 Before a contraction stress test, the nurse explains contractions will be produced by: NCLEX

- **a.** Leopold's maneuvers
- **b.** Ambulation
- **c.** Small doses of oxytocin or nipple stimulation
- **d.** Amniotomy

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Ideal time for fetal surveillance	After 24 weeks (viability in Egypt)
Fetal movement count	Daily from 28 wk; <10 in 12 h = compromise

NST timing	After 32 weeks, twice weekly
Reactive NST	≥2 accelerations ≥15 bpm × ≥15 s in 20 min → normal
BPP score	8–10 normal; ≤6 fetal compromise
BPP components	Breathing, movement, tone, amniotic fluid + NST
CST	Contractions induced by oxytocin or nipple stimulation
Baseline FHR	110–160 bpm
Bradycardia	Baseline < 110 bpm
Acceleration	Abrupt rise ≥15 bpm for ≥15 s — reassuring
Early deceleration	Head compression — mirrors contraction
Late deceleration	Uteroplacental insufficiency — after the peak
Variable deceleration	Cord compression — change position
Amniotic fluid normal	Clear/pale straw with white vernix specks
FHR 174 bpm	Tachycardia → notify provider

LECTURE 9

Physiology & Nursing Management of Normal Labor

فسيولوجيا الولادة الطبيعية ورعايتها

Part A · Past Exam Questions

أسئلة الدفعات السابقة

9.1 In a multipara, the first stage of labor usually lasts: تكرر في 2 مصادر — كويز 2 – 2022

a. 6 to 8 hours
b. 3 to 6 hours
c. 10 to 15 hours
d. 24 hours

9.2 Ripening of the cervix occurs during the: تكرر في 2 مصادر — كويز 2 – 2022

a. First stage
b. Second stage
c. Third stage
d. Fourth stage

9.3 Which of the following statements is unrelated to the signs of placental separation? تكرر في 2 مصادر — كويز 2 – 2022

a. Gush of blood
b. Uterus rises in the lower abdomen
c. Umbilical cord protrudes farther out of the vagina
d. Uterus becomes flaccid

9.4 When the fetus is in the right occiput anterior position and at +1 station, the presenting part is: — كويز 2 – 2022

a. 1 cm below the ischial spines
b. Directly in line with the ischial spines
c. 1 cm above the ischial spines
d. In no relationship to the ischial spines

9.5 The nurse is monitoring the progress of a mother in labor. Which finding should be reported to the physician immediately? — كويز 2 – 2022

a. Scant bloody discharge
b. Frequent urination
c. Green-tinged amniotic fluid
d. Moderate uterine contractions

9.6 A patient is in the active deceleration phase. How frequently should the nurse assess her uterine contractions? — كويز 2 – 2022

a. Every 5 minutes
b. Every 15 minutes
c. Every 30 minutes
d. Every 60 minutes

9.7 In the second stage of labor, expulsion of the fetus from the birth canal depends on which important contributor to the involuntary expulsive force? تكرر في 2 مصادر

كويز 2 – 2022

a. Maternal bearing down
b. Cervical dilatation
c. Uterine contractions
d. Adequate pelvic size

9.8 A nullipara is admitted with cervix 6 cm, 70% effaced, at −1 station. What phase of labor is she in? تكرر في 2 مصادر

كويز 2 – 2022

a. Active acceleration phase
b. Active deceleration phase
c. Latent phase
d. Expulsion phase

9.9 At which stage of labor is the mother advised to avoid bearing down? تكرر في 2 مصادر

كويز 2 – 2022

a. Second stage
b. First stage
c. Fourth stage
d. Third stage

9.10 A sudden gush of blood or lengthening of the cord after delivery of the infant should warn the nurse of:

كويز 2 – 2021 (نسخة ب)

a. Placenta accreta
b. Placental separation
c. Placental retention
d. Abruptio placentae

9.11 Ripening of the cervix occurs during the: تكرر في 2 مصادر

كويز 2 – 2021 (نسخة ب)

a. First stage
b. Second stage
c. Third stage
d. Fourth stage

9.12 In the second stage of labor, expulsion of the fetus from the birth canal depends on which important factor? تكرر في 2 مصادر

كويز 2 – 2021 (نسخة ب)

a. Maternal bearing down
b. Cervical dilatation and effacement
c. Uterine contractions only
d. Adequate pelvic size

9.13 A mother is in the active deceleration phase. How frequently should the nurse assess her uterine contractions?

كويز 2 – 2021 (نسخة ب)

a. Every 5 minutes
b. Every 15 minutes
c. Every 30 minutes
d. Every 60 minutes

9.14 In a multipara, the first stage of labor usually lasts: تكرر في 2 مصادر

كويز 2 – 2021 (نسخة ب)

a. 3 to 6 hours
b. 6 to 8 hours
c. 10 to 15 hours
d. 24 hours

9.15 A nullipara is admitted with cervix 6 cm, 70% effaced, at −1 station. What phase of labor is she in? تكرر في 2 مصادر

كويز 2 – 2021 (نسخة أ)

a. Active acceleration phase
b. Active deceleration phase
c. Latent phase
d. Expulsion phase

9.16 Which of the following statements is unrelated to the signs of placental separation? تكرر في 2 مصادر

كويز 2 – 2021 (نسخة أ)

a. Gush of blood
b. Uterus rises in the lower abdomen
c. Umbilical cord protrudes farther out of the vagina
d. Uterus becomes flaccid

9.17 To determine the duration of uterine contractions, the nurse notes the time from: تكرر في 2 مصادر

فاينال 2020–2019 (بنك الأسئلة)

a. The onset of the contraction until it ends
b. The beginning of one contraction until the end of the next
c. The onset of one contraction until the onset of the next
d. The end of one contraction until the end of the next

9.18 Bloody show is: تكرر في 2 مصادر

فاينال 2020–2019 (بنك الأسئلة)

a. A result of placenta previa
b. Not seen in breech presentations
c. A consequence of effacement and dilatation of the cervix
d. A sign of impending obstetric hemorrhage

9.19 Secondary forces acting to aid delivery include: تكرر في 2 مصادر

فاينال 2020–2019 (بنك الأسئلة)

a. Birth canal friction
b. Maternal bearing down
c. Cervical resistance
d. Uterine contractions

9.20 Mrs. A, a multigravida, has a cervix 7 cm dilated and 3 contractions per 10 minutes. She is in which phase? تكرر في 2 مصادر

فاينال 2020–2019 (بنك الأسئلة)

a. Latent
b. Active
c. Transition
d. Prodromal

9.21 Which assessment indicates the client is beginning the second stage of labor? تكرر في 2 مصادر

فاينال 2020–2019 (بنك الأسئلة)

a. The contractions are irregular
b. The cervix is 8 cm dilated
c. The spontaneous urge to push is initiated from perineal pressure
d. The membranes have ruptured

9.22 Show is considered a sure sign of normal labor. (True / False)

أسئلة سابقة

a. True
b. False

9.23 Labor is defined as a series of events by which the fetus and placenta are expelled from the woman's uterus. (True / False)

أسئلة سابقة

a. True
b. False

9.24 Which assessment finding indicates that the woman is beginning the second stage of labor? أسئلة سابقة

a. The spontaneous urge to push is initiated
b. The cervix is 8 cm dilated
c. The contractions are irregular
d. The membranes have ruptured

9.25 A nurse concludes that a primigravida is in the active deceleration (transition) phase without doing a vaginal examination. Which sign led to that conclusion? أسئلة سابقة

a. The woman vomited
b. The woman laughed at something
c. The woman was cooperative with the nurse
d. The woman fell asleep during a contraction

9.26 A laboring woman, G4 P3, who was 6 cm dilated 1 hour ago, cries to go to the bathroom. Which action is appropriate? أسئلة سابقة

a. Assess cervical dilation
b. Help the woman to the bathroom
c. Check the fetal heart rate
d. Ask if she needs pain medicine

9.27 At which stage of labor is the mother advised to avoid bearing down? تكرر في 2 مصادر أسئلة سابقة

a. First stage
b. Third stage
c. Fourth stage
d. Second stage

9.28 The nurse assesses the frequency of a laboring client's contractions by timing from the beginning of one contraction: أسئلة سابقة

a. Until it is completely over
b. To the end of a second contraction
c. To the beginning of the next contraction
d. Until the uterus becomes very firm

9.29 A client is admitted in early active labor. The priority nursing intervention on admission is: أسئلة سابقة

a. Auscultating the fetal heart
b. Taking an obstetric history
c. Asking when she last ate
d. Ascertaining whether membranes ruptured

9.30 A G1 P0 client is 100% effaced, 3 cm dilated, and the fetus is at +1 station. The fetal head is: أسئلة سابقة

a. Not yet engaged
b. Entering the pelvic inlet
c. Below the ischial spines
d. Visible at the vaginal opening

9.31 **Which observation indicates that the placenta has separated from the uterine wall and is ready for delivery?** أسئلة سابقة

a. The umbilical cord shortens and changes color
b. A soft and boggy uterus
c. Maternal complaints of severe cramping
d. Change in the shape of the uterus

9.32 **The nurse tells a client in early labor that effleurage is:** أسئلة سابقة

a. A form of biofeedback to enhance bearing down
b. Light stroking of the abdomen to facilitate relaxation
c. Pressure on the sacrum to relieve backache
d. Contracting a specific muscle group while others rest

9.33 **A client in the second stage of labor with contractions every 2 minutes cries out in pain with each contraction. The nurse recognizes this behavior as:** أسئلة سابقة

a. Exhaustion
b. Fear of losing control
c. Involuntary grunting
d. Valsalva's maneuver

9.34 **The fetus is documented at −1 station. The presenting part is:** أسئلة سابقة

a. 1 cm above the ischial spines
b. 1 fingerbreadth below the symphysis pubis
c. 1 inch below the coccyx
d. 1 inch below the iliac crest

9.35 **After delivery of the newborn, the nurse observes the umbilical cord lengthen and a spurt of blood from the vagina. The nurse documents these as signs of:** أسئلة سابقة

a. Hematoma
b. Placenta previa
c. Uterine atony
d. Placental separation

9.36 **A client in active labor with intact membranes is prepared for amniotomy. After this procedure she will most likely have:** أسئلة سابقة

a. Less pressure on her cervix
b. Increased efficiency of contractions
c. Decreased number of contractions
d. The need for increased BP monitoring

9.37 **The nurse determines that the client is beginning the second stage of labor when:** أسئلة سابقة

a. She begins to expel clear vaginal fluid
b. The contractions are regular
c. The membranes have ruptured
d. The cervix is completely dilated

9.38 **Before initiating a prescribed IV oxytocin (Pitocin) infusion for a client in labor, the nurse ensures that which is implemented?** أسئلة سابقة

a. Complete bed rest
b. Continuous electronic fetal monitoring
c. IV infusion of antibiotics
d. A code cart at the bedside

9.39 To determine the duration of uterine contractions, the nurse notes the time from: تكرر في 2 مصادر

أسئلة سابقة

a. The onset of the contraction until it ends
b. The beginning of one contraction until the end of the next
c. The onset of one contraction until the onset of the next
d. The end of one contraction until the end of the next

9.40 Bloody show is: تكرر في 2 مصادر

أسئلة سابقة

a. A result of placenta previa
b. Not seen in breech presentations
c. A consequence of effacement and dilatation of the cervix
d. A sign of impending obstetric hemorrhage

9.41 Secondary forces acting to aid delivery include: تكرر في 2 مصادر

أسئلة سابقة

a. Birth canal friction
b. Maternal bearing down
c. Cervical resistance
d. Uterine contractions

9.42 Mrs. A, a multigravida, has a cervix 7 cm dilated and 3 contractions per 10 minutes. She is in which phase? تكرر في 2 مصادر

أسئلة سابقة

a. Latent
b. Active
c. Transition
d. Prodromal

9.43 Which assessment indicates the client is beginning the second stage of labor? تكرر في 2 مصادر

أسئلة سابقة

a. The contractions are irregular
b. The cervix is 8 cm dilated
c. The spontaneous urge to push is initiated from perineal pressure
d. The membranes have ruptured

9.44 Which of the following presentations is the most favorable for delivery?

كويز طب نسا

a. Breech
b. Transverse
c. Cephalic
d. Oblique

9.45 Which of the following changes occurs in the cervix during labor?

كويز طب نسا

a. Thickening
b. Effacement
c. Elongation
d. Firming

9.46 During which stage of labor does the cervix dilate?

كويز طب نسا

a. First stage
b. Second stage
c. Third stage
d. Fourth stage

9.47 Which hormone is primarily responsible for initiating labor (stimulating uterine contractions)? كويز طب نسا

a. Estrogen
b. Progesterone
c. Oxytocin
d. Prolactin

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

9.N1 A primigravida is 5 cm dilated with regular contractions every 3–5 minutes. She is in: NCLEX

a. Latent phase
b. Active phase
c. Transition
d. Second stage

9.N2 The second stage of labor begins with: NCLEX

a. Rupture of membranes
b. Full cervical dilatation (10 cm)
c. Crowning
d. Placental separation

9.N3 Select ALL signs of placental separation. SELECT ALL

a. Sudden gush of blood
b. Lengthening of the cord
c. Uterus becomes globular and rises
d. Uterus becomes soft and boggy
e. Fundus descends into pelvis

9.N4 Membranes rupture and the fluid is green-tinged. The nurse should: NCLEX

a. Document as normal
b. Report to the provider and assess FHR
c. Encourage pushing
d. Give oxytocin

9.N5 Contraction frequency is measured from: NCLEX

a. Beginning to end of one contraction
b. Beginning of one contraction to beginning of the next
c. End of one to beginning of the next
d. Peak to peak only

9.N6 A woman feels the urge to push at 7 cm. The nurse instructs her to: NCLEX

a. Push hard with each contraction
b. Pant/blow during contractions to avoid bearing down
c. Hold her breath
d. Lie supine

9.N7 Station +2 means the presenting part is: NCLEX

a. 2 cm above the ischial spines
b. At the ischial spines
c. 2 cm below the ischial spines
d. Crowning

9.N8 In the fourth stage of labor, the fundus is boggy. The FIRST action is: NCLEX

a. Notify the provider
b. Massage the fundus
c. Check vital signs
d. Give fluids

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Labor	Series of events expelling fetus, placenta and membranes	**First stage**	Onset of true labor → full dilatation
Latent phase	0–3 cm	**Active phase**	4–7 cm
Transition	8–10 cm; vomiting, irritability, urge to push	**Second stage**	Full dilatation → birth of baby
Third stage	Birth of baby → delivery of placenta	**Fourth stage**	First 1–2 hours after placenta
First stage duration	Primi 10–12 h; multi 6–8 h	**Primary power**	Uterine contractions
Secondary power	Maternal bearing down	**Station**	Relation of presenting part to ischial spines (– above, + below)
Signs of placental separation	Gush of blood, cord lengthens, uterus rises & becomes globular	**Show**	Blood-tinged mucus from effacement/dilatation
Avoid bearing down	First stage (before full dilatation)	**Frequency of contractions**	Beginning of one to beginning of next

Answer Key — All Lectures

الإجابات

1.1. b — Cervix

1.2. b — Round ligament

1.3. b — Fundus

1.4. a — True

1.5. c — Vestibule and release a lubricating fluid in the vagina

1.6. a — Gynecoid

1.7. d — Fallopian tubes, uterus, vagina and ovaries

1.8. a — Endometrium

1.9. b — False

1.10. b — False

1.11. d — Cervix

1.12. a — Endometrium

1.13. a — Perineum

1.14. a — Estrogen and progesterone hormones

1.15. a — Vagina

1.16. c — Uterus

1.17. d — Fallopian tube

1.18. c — Ovary

1.19. b — Isthmus

1.20. c — Ampulla

1.21. d — Infundibulum

1.22. c — Fimbriae

1.23. b — Infundibulum, ampulla, isthmus, interstitial

1.24. a — 2 innominate, sacrum and coccyx

1.25. d — Fimbriae of uterine tube

1.26. c — Ovaries

1.27. b — Cervix

1.28. d — Fimbriae of uterine tube

1.29. c — Ovaries

1.30. b — Cervix

1.31. b — Perineal body and rectum

1.32. a — Perineum

1.33. b — Vagina, cervix, uterus, fallopian tubes and ovaries

1.34. a — Estrogen and progesterone

1.35. d — Endometrium

1.N1. b — In the ampulla of the fallopian tube
The ampulla is the widest and longest part of the tube and the usual site of fertilization.

1.N2. b — Cervix with its mucus plug
The cervix and its mucus act as a barrier to foreign bodies and organisms entering the uterus.

1.N3. c — Gynecoid
The gynecoid pelvis has a rounded inlet and adequate diameters — the classic female pelvis.

1.N4. a, b, c, d — Vagina; Uterus; Fallopian tubes; Ovaries
Internal organs: vagina, uterus, tubes and ovaries. Labia and Bartholin's glands are external genitalia.

1.N5. b — The vaginal opening and the anus
The perineal body lies between the vaginal orifice anteriorly and the anal canal posteriorly.

1.N6. b — Myometrium
The myometrium is the thick smooth-muscle layer that contracts during labor and compresses vessels after birth.

1.N7. c — Round ligament
The round ligaments pull the fundus forward, maintaining anteversion.

1.N8. b — Lubricates the vaginal introitus
Bartholin's glands open into the vestibule and lubricate the vagina during sexual arousal.

2.1. d — Decrease in both estrogen and progesterone

2.2. b — The middle

2.3. b — FSH

2.4. c — Ovum, zygote, embryo, fetus

2.5. c — Uterine tube

2.6. d — Progesterone

2.7. a — Cleavage

2.8. b — Placenta

2.9. a — Ampulla of the oviduct

2.10. a — Ovulation

2.11. c — Thickening of the endometrium

2.12. d — Progesterone

2.13. b — False

2.14. a — True

2.15. b — Trophoblast and inner cell mass

2.16. b — Placenta

2.17. c — In the 2nd week
Implantation starts about day 6–7 after fertilization and is completed during the 2nd week. (The marked answer on the original screenshot was 3rd week.)

2.18. b — The middle of the menstrual cycle

2.19. a — True

2.20. b — FSH

2.21. b — False

2.22. b — Progesterone

2.23. a — 6–8 days after fertilization

2.24. c — LH

2.25. c — Fertilization

2.26. d — FSH

2.27. b — Thin, clear and elastic

2.28. c — Follicular phase, ovulation phase, luteal phase

2.29. d — FSH

2.30. b — Thin, clear and elastic

2.31. c — Follicular phase, ovulation phase, luteal phase

2.32. b — False

2.33. a — The corpus luteum stops producing hormones

2.34. a — True

2.35. b — FSH

2.36. a — FSH and LH are released from the anterior pituitary gland

2.37. b — Two umbilical arteries and one vein

2.38. c — Ampulla

2.39. b — About 3 or 4 days after fertilization

2.40. c — Zygote stage

2.41. a — Menarche

2.42. c — The proliferative phase

2.43. b — Day 14

2.44. d — Decrease in both estrogen and progesterone

2.45. b — Anterior pituitary

2.N1. b — Around day 14
Ovulation occurs about 14 days before the next menses; in a 28-day cycle that is about day 14.

2.N2. c — LH
A mid-cycle LH surge causes rupture of the mature Graafian follicle.

2.N3. b — Progesterone
Progesterone from the corpus luteum thickens mucus; estrogen makes it thin, clear and stretchy (spinnbarkeit) before ovulation.

2.N4. a, b, d — Endometrial glands become secretory; Basal body temperature rises; Endometrium is maintained for implantation
Progesterone prepares and maintains the endometrium and raises basal temperature; thin mucus is an estrogen effect.

2.N5. b — The corpus luteum degenerates and estrogen and progesterone fall
Without hCG the corpus luteum regresses about 14 days after ovulation, hormone levels fall and the functional endometrium sheds.

2.N6. b — 3–4 days after fertilization
The morula enters the uterus about day 3–4, and implantation starts around day 6–7.

2.N7. b — It secretes mainly progesterone (and some estrogen)
The corpus luteum forms from the ruptured follicle and secretes progesterone; in pregnancy it is maintained by hCG.

2.N8. a — Trophoblast and inner cell mass
The trophoblast forms the placenta/chorion; the inner cell mass forms the embryo.

3.1. a — True

3.2. d — 3–6 months before the conceiving attempt

3.3. b — Encourage healthy lifestyles for couples desiring pregnancy

3.4. b — False

3.5. a — True

3.6. b — False

3.7. a — True

3.8. a — True

3.9. a — True
The course answer key marks this True. Note: the usual dose for low-risk women is 0.4 mg/day; 4 mg is for women with a previous neural-tube-defect pregnancy.

3.10. b — False

3.11. b — False

3.12. b — False

3.13. a — True
The course answer key marks this True. Note: the usual dose for low-risk women is 0.4 mg/day; 4 mg is for women with a previous neural-tube-defect pregnancy.

3.14. b — Encourage healthy lifestyles for couples desiring pregnancy

3.N1. b — 3–6 months before a conception attempt
3–6 months allows risks to be reduced and conditions controlled before conception.

3.N2. b — Neural tube defects
Folic acid taken about 3 months before pregnancy prevents 50–70% of neural tube defects.

3.N3. a, b, c, d — Risk assessment; Health promotion; Immunization (e.g., rubella); Management of chronic disease
NST is an antenatal fetal test, not preconception care.

3.N4. b — Achieve good glycemic control before conception
Hyperglycemia at conception increases miscarriage and congenital anomaly risk.

3.N5. c — 18 months
An interval of at least 18 months helps optimize birth outcomes.

3.N6. c — Rubella (MMR)
Live vaccines such as MMR are contraindicated in pregnancy; avoid conception for about 1 month after.

3.N7. b — It helps me become as healthy as possible before pregnancy
Preconception care is for all women of reproductive age, not only those with problems.

3.N8. b — A barrier to preconception care
Lack of services/providers is one of the listed barriers.

4.1. b — False

4.2. a — Increased systemic vascular resistance

4.3. a — True

4.4. c — Hyperplasia
Early uterine growth is due to hyperplasia (estrogen effect); later growth is mainly hypertrophy and stretching.

4.5. a — Progesterone

4.6. b — Frequency

4.7. a — True

4.8. b — Fundal height

4.9. c — Thick cervical mucus

4.10. b — The fundus of the uterus is high, pushing the diaphragm upwards

4.11. b — False

4.12. a — Increased systemic vascular resistance

4.13. d — Cheeks, forehead and nose

4.14. c — More plasma than erythrocytes

4.15. a — Increased systemic vascular resistance

4.16. a — To prevent compression of the vena cava

4.17. a — Dependent edema

4.18. d — Cheeks, forehead and nose

4.19. c — More plasma than erythrocytes

4.N1. b — Turn her onto her left side
Supine hypotension results from uterine compression of the inferior vena cava; left lateral position relieves it.

4.N2. b — Hemodilution (plasma increases more than RBCs)
Plasma volume rises about 40–50% versus RBC mass about 20–30%, causing physiologic anemia.

4.N3. a, b, c, e — Increased cardiac output; Increased blood volume; Decreased systemic vascular resistance; Slight rise in heart rate
Progesterone decreases systemic vascular resistance; an increase is NOT a normal change.

4.N4. b — Linea nigra
Linea nigra is pigmentation between the umbilicus and symphysis pubis.

4.N5. b — Pressure of the enlarging uterus on the bladder
The enlarging uterus in the pelvis presses on the bladder; frequency returns in the third trimester with lightening.

4.N6. a — Shortness of breath
The fundus pushes the diaphragm upward; headache, visual changes and epigastric pain suggest preeclampsia.

4.N7. b — Increased estrogen
Estrogen increases vascularity and mucus production.

4.N8. b — Hyperplasia stimulated by estrogen
Early growth is hyperplasia; later growth is hypertrophy and stretching.

5.1. d — Autonomy

5.2. a — Ethical dilemma

5.3. b — False

5.4. c — Justice

5.5. a — Utilitarian approach

5.6. c — Incident reports

5.7. a — Job description

5.8. a — Paternalism

5.9. c — Non-maleficence

5.10. c — When a patient authorizes disclosing it

5.11. b — Ethical principles

5.12. a — Autonomy

5.13. b — Incident reports

5.14. a — Utilitarian approach

5.15. c — Incident reports

5.N1. b — Respect her decision (autonomy) and document
A competent woman has the right to refuse treatment — autonomy.

5.N2. b — Witness the signature and confirm the client understood
The provider explains; the nurse witnesses and verifies understanding. Amniocentesis requires consent.

5.N3. a, b, d — Discussing the case only with the care team; Logging off the computer; Releasing information only with written consent
Information is owned by the patient and released only with written permission.

5.N4. b — Veracity
Veracity means telling the truth and disclosing accurate risks and benefits.

5.N5. b — Medication problems
Medication errors (dose, route, time, identification) are a common cause of lawsuits.

5.N6. b — Utilitarian approach
Utilitarian = greatest balance of benefits over risks.

5.N7. a — Fidelity
Fidelity = faithfulness/keeping promises.

5.N8. b — She may refuse but must ensure the client receives care
The ANA code upholds the nurse's right to refuse, while patient care must not be abandoned.

6.1. d — Positive ultrasound

6.2. b — Passive movement of the unengaged fetus

6.3. a — True

6.4. c — Assess the cervix for color

6.5. b — False

6.6. a — Uterine enlargement

6.7. d — Chadwick's sign

6.8. a — True

6.9. d — Chadwick's sign

6.10. d — Braxton Hicks contractions

6.11. b — Goodell's sign

6.12. c — Chadwick's sign

6.13. b — Quickening

6.14. b — It could be from implantation of the zygote

6.15. b — Chadwick's sign

6.16. b — It is the fetal movement that is felt by the mother

6.17. c — Inform her that these contractions may occur throughout pregnancy

6.18. d — Initiate a gentle upward tap on the cervix

6.19. d — Braxton Hicks contractions

6.20. b — Goodell's sign

6.21. c — Chadwick's sign

6.22. b — Quickening

6.23. a — Softening of the cervix

6.N1. c — Fetal heart tones by Doppler
Positive signs relate to the fetus: FHR, fetal movement felt by examiner, ultrasound visualization.

6.N2. b — Hegar's sign
Hegar's = soft isthmus; Goodell's = soft cervix; Chadwick's = bluish cervix.

6.N3. a, b, c, d — Amenorrhea; Nausea and vomiting; Breast tenderness; Quickening
Ballottement and hCG test are probable (objective) signs.

6.N4. a — Hydatidiform (molar) pregnancy
Molar pregnancy, trophoblastic neoplasms, pseudocyesis, user error and fertility drugs.

6.N5. c — 20 weeks
Doppler detects FHR at 10–12 weeks; fetoscope at about 20 weeks.

6.N6. b — 16 weeks
Multigravidas may notice quickening from about 16 weeks; primigravidas often at 20–25 weeks.

6.N7. b — Ballottement
Ballottement is a probable sign (internal about 16 weeks).

6.N8. b — The fetal heart rate
Funic soufflé = fetal blood through the cord; uterine soufflé = maternal pulse.

7.1. d — Leaking amniotic fluid

7.2. a — Uterus in the pelvis

7.3. c — 36 weeks

7.4. c — G5 T2 P1 A1 L4

7.5. b — Number of times a female has been pregnant

7.6. c — Nov. 7
Naegele's rule: Jan 30 − 3 months + 7 days = Nov 6. Nov 7 is the closest option (the marked answer 'Oct. 7' on the screenshot is wrong).

7.7. a — Backache

7.8. c — June 17, 2020

7.9. d — 4 weeks

7.10. d — Wash the breasts with warm water and keep them dry

7.11. c — June 26, 2014

7.12. b — G2 T0 P1 A0 L1

7.13. c — June 17, 2020

7.14. d — 4 weeks

7.15. b — Fetal heart rate of 180 bpm

7.16. c — Hemorrhage
The original form key marked 'low self-esteem' (and the form's comment says the 'loud mouth' option is a joke). Clinically, anemia increases the risk of hemorrhage complications.

7.17. b — False

7.N1. a — December 17
Subtract 3 months (December) and add 7 days (17).

7.N2. a — G4 T1 P1 A1 L3
Twins count as one birth (P1) but two living children; current pregnancy counts in G.

7.N3. a, b, c, d — Vaginal bleeding; Leaking of amniotic fluid; Severe headache with blurred vision; Decreased fetal movement
Mild dependent edema and nasal stuffiness are common discomforts.

7.N4. b — Every 2 weeks
Usual schedule: every 4 weeks until 28, every 2 weeks 28–36, weekly after 36.

7.N5. c — 20 weeks
Symphysis 12 wk, umbilicus about 20 wk, xiphoid about 36 wk.

7.N6. b — Low, broad-heeled shoes and good posture/pelvic tilt
Good body mechanics, pelvic rocking and low broad heels reduce lordosis strain.

7.N7. b — Eat dry crackers before rising and small frequent meals
Dry carbohydrates before rising and small frequent meals reduce nausea.

7.N8. a — Blood group and Rh, CBC, urinalysis
Baseline labs include blood group/Rh, Hb, urine and infection screening.

8.1. a — Normal

8.2. b — Amniotic fluid volume and fetal activity

8.3. a — True

8.4. a — The uterus is stimulated to contract by small amounts of oxytocin or nipple stimulation

8.5. c — Notify the health-care provider

8.6. a — Inform the mother that the rate is normal

8.7. a — Inform the mother that the rate is normal

8.8. b — False

8.9. b — False

8.10. a — True

8.11. a — True

8.12. a — Uterine contractions are stimulated by small amounts of oxytocin

8.13. b — Notify the health care provider

8.14. d — Surrounds, cushions and protects the fetus

8.15. a — Administer oxygen via face mask

8.16. d — Reposition the catheter, recheck, and if it is 55% keep monitoring

8.17. c — Clear, almost colorless, with little white specks

8.18. b — Change the client's position

8.19. c — Below the umbilicus on the right side

8.20. a — An acceleration

8.21. c — So that each fetal heart rate is monitored separately

8.22. b — Assessing the baseline fetal heart rate

8.23. b — Variable decelerations

8.24. d — Notify the physician or nurse-midwife

8.25. a — Document the findings and tell the mother the monitor indicates fetal well-being

8.26. d — Palpating the maternal radial pulse while listening to the fetal heart rate

8.27. b — The ultrasound shows the blood flow through the umbilical cord
Ultrasound before amniocentesis locates the placenta, fetus and amniotic fluid pockets; it is not done to show cord blood flow.

8.28. c — 150 bpm

8.N1. b — Reactive (reassuring)
Reactive NST = ≥2 accelerations ≥15 bpm × ≥15 s within 20 min.

8.N2. b — Fetal compromise — notify provider
8–10 normal; 6 or below indicates fetal compromise.

8.N3. b — Turn the woman to her left side
Intrauterine resuscitation: lateral position, stop oxytocin, oxygen, IV fluids, notify.

8.N4. a, b, c, d, e — Fetal breathing movements; Fetal body movements; Fetal tone; Amniotic fluid volume; Reactive NST
BPP = 4 ultrasound variables + NST; maternal BP is not included.

8.N5. c — Umbilical cord compression
Variable = cord compression; early = head compression; late = placental insufficiency.

8.N6. b — 110–160 bpm
Below 110 is bradycardia; above 160 is tachycardia.

8.N7. b — Fewer than 10 movements in 12 hours
<10 kicks in 12 hours (or <3 per hour) needs evaluation.

8.N8. c — Small doses of oxytocin or nipple stimulation
CST evaluates FHR response to contractions induced by oxytocin or nipple stimulation.

9.1. a — 6 to 8 hours

9.2. a — First stage

9.3. d — Uterus becomes flaccid

9.4. a — 1 cm below the ischial spines

9.5. c — Green-tinged amniotic fluid

9.6. b — Every 15 minutes

9.7. a — Maternal bearing down

9.8. a — Active acceleration phase

9.9. b — First stage

9.10. b — Placental separation

9.11. a — First stage

9.12. a — Maternal bearing down

9.13. b — Every 15 minutes

9.14. b — 6 to 8 hours

9.15. a — Active acceleration phase

9.16. d — Uterus becomes flaccid

9.17. a — The onset of the contraction until it ends

9.18. c — A consequence of effacement and dilatation of the cervix

9.19. b — Maternal bearing down

9.20. b — Active

9.21. c — The spontaneous urge to push is initiated from perineal pressure

9.22. a — True

9.23. a — True

9.24. a — The spontaneous urge to push is initiated

9.25. a — The woman vomited

9.26. a — Assess cervical dilation

9.27. a — First stage

9.28. c — To the beginning of the next contraction

9.29. a — Auscultating the fetal heart

9.30. c — Below the ischial spines

9.31. d — Change in the shape of the uterus

9.32. b — Light stroking of the abdomen to facilitate relaxation

9.33. b — Fear of losing control

9.34. a — 1 cm above the ischial spines

9.35. d — Placental separation

9.36. b — Increased efficiency of contractions

9.37. d — The cervix is completely dilated

9.38. b — Continuous electronic fetal monitoring

9.39. a — The onset of the contraction until it ends

9.40. c — A consequence of effacement and dilatation of the cervix

9.41. b — Maternal bearing down

9.42. b — Active

9.43. c — The spontaneous urge to push is initiated from perineal pressure

9.44. c — Cephalic

9.45. b — Effacement

9.46. a — First stage

9.47. c — Oxytocin

9.N1. b — Active phase
Active phase 4–7 cm; transition 8–10 cm.

9.N2. b — Full cervical dilatation (10 cm)
2nd stage = full dilatation to birth of the baby.

9.N3. a, b, c — Sudden gush of blood; Lengthening of the cord; Uterus becomes globular and rises
A boggy uterus indicates atony, not separation.

9.N4. b — Report to the provider and assess FHR
Meconium-stained fluid may indicate fetal distress.

9.N5. b — Beginning of one contraction to beginning of the next
Duration = beginning to end of one contraction; frequency = beginning to beginning.

9.N6. b — Pant/blow during contractions to avoid bearing down
Pushing before full dilatation causes cervical edema and lacerations.

9.N7. c — 2 cm below the ischial spines
Minus = above spines, 0 = engaged at spines, plus = below.

9.N8. b — Massage the fundus
Fundal massage stimulates contraction to control bleeding.